

HIV: PRE-EXPOSURE PROPHYLAXIS (PrEP)

If a HIV negative patient is at high risk of HIV infection or requests PrEP, use daily Pre-Exposure Prophylaxis (PrEP) to prevent patient from getting HIV. Continue to give condoms and lubricants as well.

Assess need for PrEP: is patient requesting PrEP?

Yes

- Without a condom
- With more than one partner

- With an HIV-positive partner (especially if partner not on ART/not virally suppressed)
- With a partner whose HIV status is unknown

- While under the influence of alcohol and drugs
- With a partner who is more than 5 years older than him/her

Yes to one or more

No to all

Has patient been recently or frequently diagnosed with STI/s?

Yes

Consider as high risk for HIV infection

Continue to advise and assess eligibility

No: consider as **low risk for HIV infection** - no need to give PrEP.

- Discuss safe sex: use condoms reliably, only 1 partner, partner testing for HIV. Offer referral for male circumcision.
- If sexually active, advise to re-test for HIV 6-12 monthly. Educate about availability of post-exposure prophylaxis.
- Encourage prompt treatment of STI for patient and partner.

Advise and assess eligibility of the patient needing or requesting HIV PrEP

- Explain that HIV PrEP is two medications in one tablet and needs to be taken every day to be effective. It helps to prevent HIV-negative people from getting HIV.
- Advise that HIV PrEP should be used together with other prevention methods (condoms, lubricants, male circumcision and contraception).
- Explain that regular HIV testing is needed: HIV test will be before starting PrEP, after 1 month and then 3 monthly.
- If pregnant or breastfeeding, explain that benefits of taking PrEP outweigh risk of any possible harm to mother or baby.

Is patient willing and ready to start HIV PrEP, to adhere to medication and to return for follow-up visits and 3-monthly HIV tests?

Yes

Test for HIV **110** (if not already done at this visit). Manage according to result:

HIV negative

Was patient at risk of HIV infection in the past 6 weeks (new or multiple sexual partner/s, or unprotected sex)?

No

Yes

Does patient have any of: unwell, poor appetite, body pain, headache, sore throat, enlarged tender lymph nodes, rash, fever, sweating?

No

Acute HIV infection unlikely

Yes

Acute HIV infection possible

- If < 30kg or on medications that affect the kidneys (e.g. amikacin, gentamicin), discuss with doctor or HIV hotline **178**.
- If history of kidney disease, do creatinine before starting PrEP: if abnormal, avoid starting PrEP.
- Take blood for creatinine and hepatitis B surface antigen and continue to assess for HIV PrEP **107**.
- Results do not have to be available to start patient on PrEP. Ensure correct contact details.

- Manage as on symptom pages.
- Repeat HIV test after 4 weeks.
- Delay giving HIV PrEP until confirmed HIV negative at 4 weeks.

HIV positive

- Avoid giving HIV PrEP.
- Explain the benefits of starting ART.
- Give routine HIV care **111**.

No

- Encourage patient to follow safe sex practices.
- Advise patient and partner to test regularly for HIV (at least 6 monthly).
- Advise patient to return once willing and ready to start HIV PrEP.